



Fiona Stanley Fremantle Hospitals Group

Sensory / calming rooms and equipment; safe and therapeutic use

Sensory rooms and equipment; safe and therapeutic use

Reference #: FSFH-MENH-GUI-0006

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health Service	Medical, Nursing, Allied Health, Patient Support Services and Facilities management

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1. Introduction

Sensory approaches and equipment are an aspect of Safewards interventions aimed to support patients who are becoming tense and/or agitated to use their own strengths and coping mechanisms to help them calm down. The Mental Health Service (MHS) promotes using sensory approaches and equipment to assist each patient to understand their individual sensory needs and develop strategies to experience increased self-awareness and improve functioning in multiple life domains.

Timely intervention using sensory approaches assists in minimising and managing unpredictable behaviour and forms part of the MHS comprehensive approach to reducing seclusion and restraint. Staff will consider sensory and or calming interventions prior to medication, where appropriate. This guideline provides procedural guidance to staff on the safe and therapeutic use of sensory rooms and sensory equipment, including the calm down methods box.

2. Terminology

Cleaning	The removal of visible soil, inorganic and organic contamination from devices or surfaces using the physical action of scrubbing with either a surfactant/detergent, water or with appropriate chemical agents.
Disinfection	Destruction of pathogenic and other kinds of micro-organisms by thermal or chemical means.
Level of alertness/emotional arousal	Rating scale from 1-10 which enables a patient to score their current level of arousal and communicate to others how they are feeling and how much support they may need. It assists the clinician and the patient to identify appropriate sensory modulation interventions for the patient to modulate their arousal level back to an optimal range.
Overstimulation	When a patient experiences an abundance of stimuli that is intolerable to them and causes confusion and/or anxiety.
Sensory modulation	The ability of the nervous system to regulate, organise and prioritise incoming sensory information in a graded adaptive manner allowing the individual to achieve and maintain an optimal range of performance and adapt to challenges in daily life.
Sensory equipment	Equipment designed to offer variable stimulation of the sensory systems – visual, auditory, tactile, olfactory, gustatory, vestibular (movement), proprioception (deep pressure).

Sensory room / Comfort room / Quiet room	A quiet, low stimulus therapeutic space where equipment and activities can be introduced to provide sensory input for calming, alerting and grounding
Calm down methods box	Portable box containing sensory equipment that can be accessed and used by patients outside of the Sensory room.
Restraint	The restriction of an individuals freedom of movement by physical or mechanical means.
Seclusion	The confinement of a person who is being provided with treatment or care at an authorised hospital by leaving the person alone in a room or area from which it is not within their control to leave: Also applies to situations where the person agrees to or requests confinement but cannot leave of their own accord.

3. Procedure

3.1. Sensory rooms

- Each mental health inpatient ward (excluding Mother and Baby Unit and Hospital in the Home (HiTH) Teams) has a sensory room which is available to patients at any time. Some boundaries will be set around access overnight to encourage sleep hygiene however the room is always available to be used as a preventative option.
- Sensory rooms will remain locked while not in use, unless otherwise determined by the individual ward.
- An up-to-date inventory of sensory equipment will be displayed in the room and patients may request items which may be obtained by a staff member if assessed as clinically appropriate for the patient to use.
- Identified sensory equipment will be stored in a locked cupboard/room if it is assessed these items may be used inappropriately or unsafely.
- Equipment should not be removed from the room unless it is stored outside the room due to limited secured storage or for cleaning purposes.
- Individual wards may allow additional items identified as therapeutic to be added to the sensory room equipment inventory following appropriate assessment by ward staff and with Nurse Unit Manager (NUM)/Clinical Nurse Specialist (CNS) approval.
- The ward Occupational Therapist (OT) and Infection Prevention and Control (IPC) Team will be consulted prior to purchase and use of new equipment.
- Individual specialist intervention including sensory assessment and prescription of sensory modalities are to be provided by OT staff as required.

3.2. Education and training

- All staff are responsible for ensuring a safe environment and should comply with these guidelines and the South Metropolitan Health Service (SMHS) Work Health and Safety Policy when using the sensory room and sensory equipment.
- The ward OT may provide training to individual clinicians on sensory modulation and/or specific equipment.
- Staff can access a free online course on sensory modulation in mental health settings [here](#).

3.3. Sensory equipment

- All sensory equipment must be assessed annually and tested as per local Facilities Management processes where required.
- Each ward will maintain a checklist of sensory equipment identifying whether the item is general access or requires secure storage. For an example of a Sensory equipment inventory checklist for general access in the sensory room see [Appendix 1](#).
- Inpatient wards will have access to a calm down methods box for patient use. See Appendix 2.
- The following sensory equipment will be securely stored (locked cupboard) in the sensory room when not in use:
 - Small bean bags – hand held. See [Appendix 3](#).
 - Weighted blanket. See [Appendix 4](#).
 - Weighted shoulder wrap. See [Appendix 5](#).
 - Aroma diffuser and scented oils (peppermint, citrus and lavender). See [Appendix 6](#).
 - Hand creams – dispensed in single amounts
 - Slinky spring
 - Vibrating hand held massager
 - Foot massager pods
 - Battery operated equipment
- Scented/Essential oils where available must be entered into Chemaalert.
- Sensory equipment for HiTH and community teams will be stored in the team office and utilised with patients under supervision of the visiting clinician and guidance of the Senior OT. It will be cleaned and returned to the team office after each visit
- Sensory equipment for the Mother and Baby unit will be securely stored in the sensory cupboard in the office when not in use. Low risk sensory items are also located on the ward in the activity room and can be requested as necessary.

3.4. Patient assessment

- A staff member will conduct and document an assessment of the patient's mental health risk prior to use of the sensory room and approve the patient as suitable.
- Patients with complex co-morbidities (cardiac/respiratory, pregnancy, age-related issues, significant cognitive impairment, epilepsy, musculoskeletal concerns, claustrophobia) must have a further risk assessment conducted by the treating doctor prior to using the equipment
- Certain equipment and techniques may only be used by practitioners with training in their safe use for example weighted blanket and essential oils. Refer to Appendices for instructions on the use of individual equipment.
- Exclusion criteria for the sensory room and communal sensory equipment:
 - Scabies/lice infestations
 - Current viral, bacterial or fungal infections
 - Unexplained, acute onset of fever or fever with accompanying cough
 - Vomiting or diarrhea
 - Skin abscesses, boils or open wounds
 - Other conditions not listed posing an infectious risk
- Exclusion criteria is for the duration of illness listed above.
- In the event of an infection outbreak certain equipment may be removed after consultation with IPC.

3.5. Using the sensory room

- Once the patient is assessed as suitable to use the sensory room the Shift Coordinator will be informed and the staff member will ensure the room is opened.
- The staff member will also provide orientation and instruction on expected behaviour and the desired outcome for the patient.
- The patient may leave the sensory room or stop using the sensory equipment at any point in time they choose. The recommended length of time for use of the sensory room is customised to meet the patients' needs e.g. 5 – 30 minute intervals.
- Staff must document the patients use of the sensory room in their medical record. Where appropriate documentation will include:
 - Time in
 - Time out
 - Patients self-rated level of alertness/emotional arousal before and after use
 - Staff rating of the patients level of alertness/emotional arousal before and after use

- Patients will be observed at least every 15 minutes, more frequently if appropriate and will be monitored for signs of overstimulation and adverse effects. Referr to Appendix 6 Supervision Levels for Equipment.
- Where closed circuit television is available it can be used to monitor the patient while utilising the room. Staff observations should be as unobtrusive as possible.
- In the youth/adult setting only one patient at a time is permitted to access the sensory room unless as part of a therapeutic group intervention.
- In the older adult setting:
 - it may be appropriate for more than one patient to use the room following a risk assessment (e.g. knitting together).
 - patients assessed as appropriate by the Shift Coordinator/ nursing staff may use of the sensory room for respite from overstimulation in the social area. In this situation the patient will be set up in the sensory room with suitable staff supervision.
- Sensory equipment used for an individual patient will be based on patient requests and coping strategies identified in the Treatment Support and Discharge Plan (TSDP) or at staff discretion
- Food , drink or personal items are not permitted in the room unless they are specified as an intervention in the TSDP or as a medical directive e.g. meal support
- The sensory room inventory of equipment will be checked fortnightly and missing or damaged equipment is to be escalated

Site	Checked by	Escalated to
FSH	Nursing / delegated staff member	NUM/CNS
FH	OT Assistant	Senior OT

- Any damaged equipment should be removed and local procedures for the initiation of repairs immediately followed.

3.6. Cleaning

- The staff member who facilitated access to the sensory room is responsible for ensuring the space is left in a clean and orderly state and equipment is cleaned as per the following FSFHG policies:
 - Standard and Transmission Based Precautions
 - Infection Prevention & Management Equipment Risk Assessment
 - Environmental Cleaning – Fiona Stanley Hospital
 - Environmental Cleaning – Fremantle Hospital

- Staff should always follow the manufacturer's instructions on cleaning items. All items of equipment require cleaning after use and must be allowed to air dry before being used by the next patient.
- Equipment will be cleaned using either:
 - Neutral detergent
 - Hospital approved wipes. Equipment must be assessed as compatible before use.
- Refer to appendices for cleaning regimens for specific equipment.
- A patient must be requested to attend hand hygiene on entering and exiting the sensory room using either:
 - Alcohol based hand rub
 - Hand washing with soap and water

4. Compliance/Performance Monitoring

Any clinical incident related to the use of sensory rooms and / or sensory equipment will be reported through the Datix Clinical Incident Management System. The ward NUM / CNS will monitor compliance with this policy and procedure via routine clinical incident review.

The sensory room inventory will be checked fortnightly by a ward nominated delegate to ensure all equipment is available, clean and in good working order. The fortnightly checks will be audited on a three monthly basis and results can be tabled at the local Towards Eliminating Restrictive Practices meeting as required.

5. Related Policy Documents

SMHS:

- [Electrical equipment: general information SMHS: 154](#)
- [Managing Unsafe Equipment](#)
- [WorkSafe Inspection, Notices and Safety Alerts](#)
- [Safe Work Procedure](#)

FSFHG:

- Restrictive Practices (FSFH-MENH-POL-0019)
- Infection prevention & Management Equipment Risk Assessment (FSH-HW-POL-0037)
- Hand hygiene and Skin Integrity (FSH-IPM-POL-0004)
- Standard and Transmission Based Precautions (FSFH-IPM-POL-0085)
- Linen management (FSH-HW-POL-0020)

- Environmental Cleaning FH (FSFH-HW-POL-0048)
- Environmental Cleaning (FSFH-HW-POL-0002)

6. Related Standards

NSQHS standards:

- Preventing and Controlling Infection

National Standards for Mental Health Services:

- Safety
- Consumer and Carer Participation
- Delivery of care.
- Rights and responsibilities

Chief Psychiatrists Standards for Clinical Care:

- Seclusion and Bodily Restraint Reduction

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8. Authorisation

EXECUTIVE SPONSOR: Service Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	07/2019	Towards Eliminating Restrictive Practices Working Groups	Towards Eliminating Restrictive Practices Working Groups	FSFHG Policy Committee	07/2022
2	08/2022	CNS Mental Health Service	Towards Eliminating Restrictive Practices Working Groups	Service 5 Safety Quality and Risk Committee	08/2025
3.	06/2023	CNS Mental Health Service		Service 5 Safety Quality and Risk Committee	06/2027

9. Appendices

Appendix 1. EXAMPLE Sensory room equipment inventory checklist

Ward:

Date:

Sensory room inventory to be checked fortnightly

- Use action taken to describe corrective actions e.g. cleaned equipment, reported to NUM, or requested ward clerk to reorder stock
- Weighted blanket washed: _____ Weighted shoulder wrap washed: _____

General access items	Stock	Present (✓ or x & number)	Functional (✓ or x)	Hygienic (✓ or x)	Action taken	Name	Signature
Recliner chair	1						
Bean bag chair	1						
Sensory pillow	1						
TV, DVD player	1						
Radio/ stereo	1						
Fleece blanket	1						
Stretch caterpillar	1						
Puffer(sensory) ball	1						
Rubik's cube	1						
Squeeze ball –atomic bead	2						
Complete Calm DVD	1						
AquaZone DVD	1						
Hidden Forest CD	1						
Kakadu CD	1						
Summer Rain CD	1						
Memento CD	1						
Activity file with handouts	1						
Locked cabinet items	Stock	Present (✓ or x & number)	Functional (✓ or x)	Hygienic (✓ or x)	Action taken	Name	Signature
Weighted blanket (5.2kg)	1						
Extra cover - Weighted blanket	1						
Weighted shoulder wrap	1						
Star Projector 2013	1						
Light up jellyfish mini lamp	1						
Bubbles (single use only)	1 bottle						
Black pens, Colour pencils, Felt tip pens	1 pkt each						
Sorbelene cream	1						

Bean bags	4						
Fragrance Oil – Peppermint, Orange, Lavender 250ml	1 each						
Aroma diffuser & cotton wool pads	1						
Vibrating portable Massager	1						
Plastic slinky spring	1						
Foot pods	4						

Appendix 2: Calm Down Methods Box

EQUIPMENT: Calm Down Methods Box		
HAZARDS: Choking, suffocation, accidental injury i.e. bruising	RISK RATING:	MODERATE

THERAPEUTIC USE

- Inpatient wards will have access to a calm down methods box for patient use
- Sensory equipment provided may include:
 - Laminated sheet listing possible calming strategies
 - Stress balls to squeeze – spiky or foam
 - Bouncy ball (large –hand size)
 - Hard massage balls or rollers
 - Relaxation CDs e.g. Tony O'Connor – Hidden forest, Kakadu, Summer rain, Memento
 - Disposable ear plugs to block out noise
 - Sweet/sour lollies for sucking (portion controlled, sugar free, individually sealed and for single patient use only)
 - Herbal teas (individually sealed and for single patient use only)
 - Scented hand creams
 - Scented pillows e.g. lavender bags
 - Weighted animal or weighted lap bag
 - Activities to complete e.g. Zentangles, oil pastels, colour by numbers, crosswords
 - A book with positive sayings/thoughts
 - Pack of cards
 - Small squares of bubble wrap
 - Breathing/relaxation instructions
 - Lighting option e.g. 'underwater volcano' or liquid timer/spiral



- To augment the use of the calm down safe methods box, calming strategies may be offered including:
 - having a carbohydrate snack e.g. toast, biscuits yoghurt
 - 15 minutes of staff time
 - going for a walk outside the ward or in the garden/courtyard if able
 - watching television
 - reading poetry
 - talking to other patients
 - distraction by playing video games or engaging in games such as cards, scrabble
 - calling family or friends by phone
 - artwork
 - time in bedroom or quiet area
 - having a shower
 - exercise with or without multidisciplinary team (MDT) supervision such as jogging laps, push ups
 - relaxation and deep breathing exercises which MDT staff can talk through and practise with the patient
 - picture books
 - holding and feeling objects with different textures

The calm down method box will be assigned to supervision Level B/C according to equipment in use.

THERAPEUTIC RISK

The treating team will complete a risk assessment of the patient to determine:

- The patients suitability for using equipment
- The patient's safety while using the equipment (e.g. potential for harm to self or others).

RISKS (According to Manufacturer's Operating Manual)

- Refer to individual items.

SAFETY RULES

- The items in the Calm Down Methods box will be appropriate to the patient cohort e.g. youth, adult or older adult.
- A laminated list of the inventory of items will be clearly displayed on the box.
- Sensory equipment will be used according to Safewards guidelines

PROCEDURE

Prior to Calm Down Methods Box use

Staff will:

- assess the suitability of items in the box for the individual patient before offering to the patient
- remove items if assessed as unsuitable e.g. sweet/sour lollies
- provide education and/or a demonstration of the safe use of items including hand hygiene and infection prevention
- ensure patient understands items are on loan only

During use

Offer appropriate supervision to clients using the items by either:

- supporting patients to engage in one to one activities which can be done jointly by the patient and staff member
- observing the patient at a minimum every 15 minutes.

After use

Staff will:

- discard single use items used by the patient
- check all items are returned using the inventory checklist on the box and restock if necessary

CLEANING / INFECTION CONTROL PROCEDURES

- The box and sensory equipment must be able to be effectively cleaned between use.
- Oxivir Tb wipes (Hydrogen peroxide 0.5%) for cleaning of equipment

Appendix 3: Small bean bags

EQUIPMENT: Small bean bags		
HAZARDS: Choking, suffocation, accidental injury i.e. bruising	RISK RATING:	MODERATE

THERAPEUTIC USE

Small bean bags are used for a tapping activity which provides strong deep pressure (proprioception) input to the body.

The use of small bean bags is aimed to:

- Provide sensory input which facilitates awareness of the body/self
- Promote self-regulation through awareness of the body/self
- Provide brain feedback which assists the person to be more functional and organised
- Assist with relaxation
- Assist in regulating emotions

The bean bags will be assigned to supervision Level B/C

THERAPEUTIC RISK

The treating team will complete a risk assessment of the patient to determine:

- The patients suitability for using this equipment
- The patient's safety while using the equipment (e.g. potential for harm to self or others).

RISKS (According to Manufacturer's Operating Manual)

- Beans within small bean bags may present a choking hazard.
- Bean bags will be inspected before use.
- Ensure bean bag is not used to cover patients head or face.
- Accidental injury i.e. bruising.

SAFETY RULES

- Light tapping can be irritating and not calming.
- Never tap on the neck, face, forehead, stomach, or any other area which causes discomfort for the patient.
- The patient should not tap so vigorously that it causes irritation or pain.
- Staff will monitor the use of tapping to assess for any accidental bruising or use by the patient for self-harm.

- Stop use and seek medical advice if pain, irritation or bruising occurs.

PROCEDURE

- Provide patient with information on purpose of the small bean bag and how to use it safely.
- Encourage tapping to be done with an attitude of kindness to the body.
- Encourage hand hygiene prior to use and patient to wear clean socks.
- Provide the patient with one small bean bag, the size will depend on the person but should fit comfortably in one hand.
- Staff member to demonstrate on themselves.
- The patient will use the bean bag to 'tap' areas of the body such as arms and legs with varying rate, pressure and rhythm to find their preference.
- The taps need to be firm enough to reach deep pressure touch receptors under the skin but should not be painful.
- Start by tapping opposite hand on both the palm and back of the hand.
- Work up the arm, experimenting with tapping on various areas of the arm.
- Next tap the shoulders.
- Tap across upper chest on pectoral muscles.
- Follow up with opposite hand, arm and shoulders.
- Tap as much of the back that can be reached comfortably.
- Move onto tapping legs, thighs, knees, shins and feet.

CLEANING / INFECTION CONTROL PROCEDURES

- Wipe with hospital approved wipes and the bean bags can be reused.
- The bean bags will be made from vinyl with reinforced edging (double stitched).
- Small bean bags will be replaced every 3-6 months depending on usage.

Appendix 4: Weighted blanket

EQUIPMENT:	Weighted blanket	Supervision Level A-B
HAZARDS:	Choking, suffocation	RISK RATING: MODERATE

THERAPEUTIC USE



The aims for use include:

- To calm and relax
- To increase capacity for conversation and activity tolerance
- To increase reality orientation and environmental awareness
- To facilitate self-organisation via proprioception
- To self-soothe
- To comfort and nurture by simulating a non-threatening self-hug/feeling of being held
- As a grounding technique to use when 'in crisis', distressed, anxious or highly aroused
- To provide a sense of safety via a feeling of containment and being 'grounded' or connected to the body

The weighted blanket will be assigned to supervision Level B.

If the blanket exceeds 10% of a person's body weight or contraindications are noted, an individual assessment and intervention plan for use must be completed by a Occupational Therapist specifying the level of observation required is Level A.

THERAPEUTIC RISK

The treating team will complete a risk assessment of the patient to determine:

- The patients suitability for using this equipment
- The patient's safety while using the equipment (e.g. potential for harm to self or others).

The risk assessment will consider physical examination, patient history, weight, level; of alertness, sedation, awareness and ability to follow instructions.

RISKS (According to Manufacturer's Operating Manual)

- Pellets within the weighted pouches may present a choking hazard.
- The weighted blanket will be inspected prior to use.
- A trolley should be used for transporting the weighted blanket; it should not be carried.

SAFETY RULES

- The treating doctor must approve the use of a weighted blanket for patients with the following:
 - Pregnancy – do not use weight over the abdomen or chest
 - Cardiac/respiratory comorbidities - not for use on chest
 - Musculoskeletal conditions including fractures and back pain – position the patient carefully before applying any weight
 - Trauma history – consider use carefully; patient to control use of blanket
 - Claustrophobia and anxiety – grade use and monitor response
 - Skin integrity concerns – not for use with open wounds, care to be taken with fragile skin
- Patient observation will be in accordance with allocated supervision level for this sensory equipment.
- Do not use the blanket overnight, while the patient is sleeping or when heavily sedated.
- Do not put the blanket over the face or head.
- Patients are not to walk around when using the blanket as this increases falls risk and the risk of musculoskeletal harm.
- Weighted blankets are not for use as a form of restraint however the patient may request use of the blanket if restraint is required.

PROCEDURE

- Explain the purpose and precautions for use to the patient.
- Discuss preferred use with the patient. Promoting their choice, control and adjustment and maintain a dialogue with the patient in response to them.
- In the sitting, reclining or lying position, drape the blanket over the front of the patient – wherever they prefer the weight if not contraindicated.
- Ensure the patient has the capacity to self-control application and adjustment of the weighted blanket, if the patient cannot move the blanket readily it is too heavy for them and consider alternatives
- Some patients may choose to be wrapped in the blanket, loosely or tightly.

- Adjust the weight by having some of the blanket resting on the ground or furniture rather than on the patient, or removing individual weights.
- Use the blanket for calming as long as the patient wishes, generally 20 minute intervals. Avoid longer time frames.
- When using the blanket as a grounding technique, use for less than 20 minutes. Remove and reintroduce to maximise grounding effect and to prevent habituation.
- Patient monitoring is required:
 - When the weighted blanket exceeds 10% of patient body weight
 - If the weighted blanket is used for restraint at the patients request
 - If the blanket is indicated and approved for use outside the comfort room
 - Prior to and during use for patients with conditions listed above
- Patient monitoring includes:
 - Full set of physiological observations including temperature
 - Oxygen saturation
 - Behavioural changes
 - Patient feedback
- The weighted blanket may be used in conjunction with other sensory equipment such as relaxing music or aroma diffuser.
- If the blanket feels too warm or heavy, remove immediately.

CLEANING / INFECTION CONTROL PROCEDURES

- Spot clean any obvious soiling with hospital approved wipes.
- Shake out well after use.
- The weighted blanket cover will be used for one patient only, it may be used more than once for the same patient if no obvious soiling.
- The weighted blanket cover will be laundered in between patients in accordance with the Australian/New Zealand Standard (AS/NZS) 4146-2000 – Laundry Practice.
- Weights should be removed prior to laundering and ensure weights are replaced securely following cleaning.
- Laundering of the weighted blanket cover will be documented on the ward inventory checklist by the ward OT/OTA and monitored by the Nurse Unit Manager.

Appendix 5: Weighted shoulder wrap

EQUIPMENT:	Weighted shoulder wrap		
HAZARDS:	Choking, suffocation	RISK RATING:	MODERATE

THERAPEUTIC USE



The aims for use include:

- To calm and relax
- To increase capacity for conversation and activity tolerance
- To increase reality orientation and environmental awareness
- To assist the patient to become more organised and functional in their environment
- To self-soothe
- To comfort and nurture by simulating a non-threatening self-hug/feeling of being held
- As a grounding technique to use when 'in crisis', distressed, anxious or highly aroused
- To provide a sense of safety via a feeling of containment and being 'grounded' or connected to the body

The weighted shoulder wrap will be assigned to supervision level B.

THERAPEUTIC RISK

- The treating team will complete a risk assessment of the patient to determine:
 - The patients suitability for using this equipment
 - The patient's safety while using the equipment (e.g. potential for harm to self or others).
- The risk assessment will consider physical examination, patient history, weight, level; of alertness, sedation, awareness and ability to follow instructions.

RISKS (According to Manufacturer's Operating Manual)

- Pellets within the weighted pouches may present a choking hazard.
- The weighted shoulder wrap will be inspected prior to use.
- If contraindications are noted an individual assessment and intervention plan for use must be completed by a suitably qualified professional/Occupational Therapist.
- A trolley should be used for transporting the weighted shoulder wrap; it should not be carried.

SAFETY RULES

The treating doctor must approve the use of a weighted shoulder wrap for patients with the following:

- Pregnancy – do not use weight over the abdomen or chest
- Cardiac/respiratory comorbidities – not for use on chest
- Musculoskeletal conditions including fractures and back pain – position the patient carefully before applying any weight
- Trauma history – consider use carefully; patient to control use of wrap
- Claustrophobia and anxiety – grade use and monitor response
- Skin integrity concerns – not for use with open wounds, care to be taken with fragile

A full set of physiological observations is required to be documented including oxygen saturation prior to use.

PROCEDURE

- Explain the purpose and precautions for use to patient including not mobilising with the weighted shoulder wrap.
- Position patient in sitting, reclining or lying position.
- Drape weighted shoulder wrap over shoulders or lap of the patient, wherever they prefer the weight. Do not use over the face or head.
- Adjust the weight of the shoulder wrap to suit the patients preference by holding the panels at the front upwards to tip more or less weight over the back
- The weighted shoulder wrap can be used for calming for as long as the patient wishes, generally about 20 minutes.

If the weighted shoulder wrap feels too warm or too heavy, remove it immediately and consider alternatives.

The weighted shoulder wrap can be used in conjunction with other sensory equipment such as relaxing music or the aroma diffuser.

CLEANING / INFECTION CONTROL PROCEDURES

- Spot clean the shoulder wrap if any obvious soiling with hospital approved wipes and shake out well .
- The weighted shoulder wrap cover will be used for one patient only, it may be used more than once for the same patient if no obvious soiling.
- The weighted shoulder wrap cover will laundered in between patients in accordance with the Australian/New Zealand Standard (AS/NZS) 4146-2000 – Laundry Practice.
- Weights should be removed prior to laundering and ensure weights are replaced securely following cleaning.
- Laundering of the weighted shoulder wrap will be documented on the ward inventory checklist by the ward OT/OTA and monitored by the Nurse Unit Manager.

Appendix 6: Aroma diffuser and scented oils.

EQUIPMENT:	Aroma diffuser and scented oils		
HAZARDS:	Flammable	RISK RATING:	MODERATE

THERAPEUTIC USE

The aims for use include to:

- Relax and balance , self nurture and self-soothe
- Increase patients capacity to regulate emotions through either calming, alerting or grounding
- Promote self-regulation and self-organisation through olfactory sensory input

THERAPEUTIC RISK

- Scented oils are for external use only.
- Certain aromas may be triggering or unpleasant to some patients.
- Clinical aromatherapy is a complementary therapy which uses 100% essential oils for a specific measurable outcome. The use of essential oils will only occur after the treating team has completed a risk assessment of the patient to determine suitability.
- Essential oils will be used with caution in pregnancy and for patients with epilepsy, hypertension, oestrogen-dependent tumours, moderate to severe respiratory problems, with scent sensitivities and plant or fruit allergies.
- Some essential oils cannot be used whilst pregnant including lemongrass and peppermint.
- The use of aroma therapy diffuser will be assigned to supervision level C.

RISKS (According to Manufacturer's Operating Manual)

Essential oils are highly concentrated, flammable, fragrant liquids for use in a vaporiser.

SAFETY RULES

- Clinical aromatherapy may only be used under the direction of a qualified aromatherapist and with the patients consent as part of their agreed care plan.

- Essential oils will be stored in dark or amber glass bottles in the locked cabinet.
- Any clinical incident related to the use of the aroma diffuser will be reported through the Datix Clinical Incident Management System.

PROCEDURE

Scented oils:

- Explain the purpose and safe use to the patient.
- Before use allow the patient to smell the different scent options to identify their preferences for scent and strength.
- Place the diffuser in a safe and convenient position on a table or stable surface
- Add a small drop of the desired scented oil to a cotton pad and insert at the base of the aroma diffuser.
- Connect the diffuser to a power source and turn on.
- After use turn off the unit and remove the cotton pad from the base of the diffuser.

Essential oils:

Appropriately trained nursing and occupational therapy staff will assess the appropriateness of essential oils with the patient – refer to procedure for a guide on essential oils

Name	Aim for use	Possible benefit
Geranium	Relaxation and emotion regulation	Balances the mind and body by relieving nervous tension, stress and mild anxiety. Calming relief of pre-menstrual symptoms
Lavender	Relaxation and emotion regulation	Restores and balances mind and body by symptomatic relief of mood swings, stress, anxiety and insomnia. Relief of muscular aches and pains. Temporary relief of headaches.
Peppermint	Relaxation and emotion regulation	Aids with digestion and provides relief from sinus pain and cold symptoms. Can be effective in reducing the joint inflammation associated with arthritis and gout. Not for use in pregnancy
Lemongrass	Refreshing and stimulating mood	Refreshes and stimulates mind/body by assisting in the temporary relief of headaches and maintaining healthy digestive function. Can assist in relief of muscular aches and pains, cramps and spasms. Not for use in pregnancy.

Orange	Refreshing and stimulating mood	Pleasant, uplifting oil which restores the spirit. Aids with headache and stress. Can assist in reducing the severity of mood.
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CLEANING / INFECTION CONTROL PROCEDURES

- Aroma diffusers used in Sensory rooms will be purchased from a therapeutic equipment supplier e.g. Technical solutions www.tecsol.com.au
- Discard any left over water in laundry sink.
- Wipe the diffuser clean with hospital approved wipes and dry with a paper towel/soft cloth before storage in the locked cupboard.
- Refer to manufacturer's instructions for further information on maintenance, care and trouble shooting.



Appendix 7: Supervision levels for equipment

These supervision levels are for sensory equipment use only, the clinically ordered psychiatric level of patient observations must continue in parallel with below equipment observations.

Supervision Level	Frequency of observation
Level A	1:1
Level B	10 minutes
Level C	30 minutes
Level D	60 minutes